

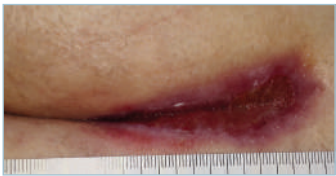
Advise the patient with a skin ulcer or non-healing wound

- Advise a healthy diet ↗ 11 and to be as active as possible. If patient smokes, advise that smoking delays healing and encourage to stop ↗ 141. Support patient to change ↗ 177.
- If **pressure ulcer**, advise to move (lift, avoid dragging) patient every 1-2 hours if unable to shift own weight.
- If **venous or diabetic ulcer**, advise to avoid prolonged standing and to elevate limb whenever possible.
- If **diabetes** or **PVD**, give foot care education ↗ 66.
- Encourage patient to adhere to chronic treatment and to aim towards achieving control of chronic conditions. If difficulty with adherence ↗ 173.
- Advise patient to keep dressing in place and not to tamper with wound/dressing. If patient instructed to change dressing, advise to wash hands with soap and water before doing so.
- Refer patient to community health worker. Also consider referral to social worker if available.

Treat the patient with a skin ulcer or non-healing wound

- If pain worse with dressing changes, give dose of patient's regular pain medication one hour before removing dressing.
- Gently remove dressing to avoid damaging healing tissue. If dressing sticks, wet with **sodium chloride 0.9%** solution and wait 5-10 minutes before gently re-attempting.
- Remove dead tissue from wound. If large areas of dead tissue or undermined wound edges, arrange for debridement. Avoid debridement if reduced blood supply or cancer wound.
- Gently clean/irrigate wound, remove any plaster debris and apply appropriate dressing, if available, according to type of wound bed:

Epithelialising wound (pink)



Source: Anne Berzen

- Avoid cleaning wound bed: clean surrounding skin only with **sodium chloride 0.9%** solution or clean tap water.
- Apply paraffin or petroleum gauze, or other non-adherent dressing.
- If wound dry, apply **hydrogel** to wound before dressing.

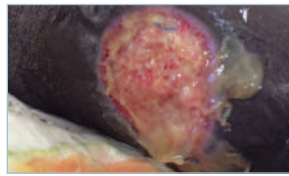
Granulating wound (red)



Source: Vanessa Lomas

- Irrigate wound and clean surrounding skin with **sodium chloride 0.9%** solution or clean tap water.
- Apply paraffin or petroleum gauze, or other non-adherent dressing.

Sloughy wound (yellow)



Source: Vanessa Lomas

- Irrigate wound and clean surrounding skin with **sodium chloride 0.9%** solution or clean tap water. If slough remains, gently remove with moistened gauze.
- If excessive discharge, apply barrier film/paste to surrounding skin.
- Apply hydrocolloid, hydrofibre or foam dressing.

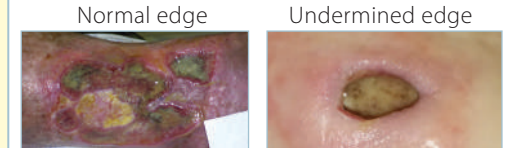
Necrotic wound (black, dry)



Source: Anne Berzen

- Apply **hydrogel** to wound, then apply paraffin or petroleum gauze, or other non-adherent dressing. Cover with film dressing if available.
- Doctor to assess need for debridement.

Infected wound



Source: Francois Coetzee

Source: Anne Berzen

- Irrigate wound and clean surrounding skin with **sodium chloride 0.9%** solution or **povidone iodine 10%** solution (up to 2 weeks only). If slough remains, gently remove with moistened gauze.
- Apply thin layer of **silver sulfadiazine** or **medicinal honey** to wound, then apply absorbant dressing.
- If excessive discharge, apply instead silver hydrofibre or alginate dressing.

- If non-adherent dressing used, cover with cling bandage. Avoid compression if reduced blood supply to limb.
- If **venous ulcer**, apply compression bandage over dressing, starting at toes (not higher on foot) and ending below knee.
 - Avoid compression bandage if reduced blood supply to limb.
 - If numbness, pins and needles, or severe pain develop, advise patient to remove compression bandage and return immediately.
- If pain, give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed. If no response, give **ibuprofen**¹ 400mg 8 hourly with food as needed.
- If **infection** likely, give **amoxicillin/clavulanic acid**² 875/125mg 12 hourly for 7 days, or 10 days if diabetic ulcer. If no better after 2-3 days, take wound swab ↗ 76. If worsens despite antibiotic, refer same day.

Review the patient with a skin ulcer or non-healing wound

- If wound **infected**: change dressing daily. If silver hydrofibre dressing used, change dressing every 2-3 days instead.
- If wound **sloughy** or **necrotic**: change dressing around every 2 days, depending on amount of discharge.
- If wound **epithelializing** or **granulating**: change dressing weekly.

If cause uncertain, recurrent ulcers or no better after 1 month, refer.

¹Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease. ²If penicillin allergy, discuss/refer.